

China Healthcare: Eyes on ADA: Innovent joining the oral GLP-1 race; Emerging portfolio synergies for Hengrui

Ahead of the American Diabetes Association (ADA) conference (June 5-8, 2026), we highlight that China biotech/pharma companies are expected to share updates on their metabolic pipeline, with particular focus on the incremental updates of the oral GLP-1 drug candidates, led by Innovent's small-molecule GLP-1s (IBI3032 and IBI3042) and Hengrui's oral peptide HRS-9531 (GLP-1/GIP).

The oral transition is underway for GLP-1, expect bigger role of China assets: We expect the GLP-1 landscape to continue evolving, with increasing role for oral GLP-1 given: 1) the recent approval of Wegovy pills and Foundayo (orforglipron), and 2) the anticipated robust position against the disruption from generic injectable semaglutide (see our most recent GLP-1 piece on [generic entrants](#)). Projected to capture c.35% share of the 2030 AOM TAM, the oral obesity pill market is on its way towards US\$35+bn by 2030 (see [note](#) from our US/EU team), led by Foundayo and Wegovy pills, with increasing investor focus on the prescription trends for both drugs ([Wegovy pill tracking beat](#), [Foundayo weekly TRx in focus](#)) in the initial ramp-up phase. Beyond the leading duo, a batch of oral GLP-1 drug candidates have entered late-stage clinical trials, with meaningful contribution from China ([Exhibit 1](#)), and we expect China-originated assets to play a bigger role in oral GLP-1 with the pipeline progress.

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Exhibit 1: We expect China assets to play a bigger role in oral GLP-1 with the pipeline progress

Landscape of oral GLP-1

GLP-1RA	MoA	Company	China Stage	Ex-China Stage	Modality	Dosage	Titration
Oral							
Wegovy pills (Oral Sema)	GLP-1	Novo Nordisk	Ph3	Approval	peptide	25mg, QD	4 steps, 12 wks, 3-25mg
orforglipron	GLP-1	Eli Lilly	NDA	Approval	small molecule	36mg, QD	6 steps, 20 wks, 2-36mg
VK2735	GLP-1/GIP	Viking Therapeutics	n.a.	Ph3	peptide	2.5-40mg, QD	
alenglipron (GSBR-1290)	GLP-1	Structure Therapeutics	IND	Ph2	small molecule	5-90mg, QD	6 steps, 6 wks, 5-120mg
ASC30	GLP-1	Ascleitis	n.a.	Ph2	small molecule	20-60mg, QD	7 steps, 6 wks, 1-60mg
elecglipton (ECC5004)	GLP-1	Eccogene/AstraZeneca	Ph1	Ph2	small molecule	10-150mg, QD	
VCT220	GLP-1	Vincetage/Corxel	Ph3	Ph2	small molecule	20/40mg, QD	
HRS-7535	GLP-1	Hengrui/Kailera	Ph3	Ph2	small molecule	15-120mg, QD	4 steps, 3 wks, 30-120mg
CT-996	GLP-1	Roche	Ph1	Ph2	small molecule	120mg, QD	5 steps, 3 wks, 10-120mg
RGT-075	GLP-1	Regor Therapeutics	n.a.	Ph2	small molecule	15-280mg, QD	
PF-07976016	GIP	Pfizer	n.a.	Ph2	small molecule		
BGM0504	GLP-1/GIP	BrightGene	Ph3	Ph1	peptide		
XW004 (Oral ecnoglutide)	GLP-1	Sciwind	Ph1/2	Ph1	peptide	7-30mg, QD; 60mg QW	
IBI3032	GLP-1	Innovent	Ph1	Ph1	small molecule	QD	
zenagamtide (Oral amycretin)	GLP-1/amylin	Novo Nordisk	Ph1	Ph1	peptide	50mg, QD	
YP05002	GLP-1	Pfizer/Fosun	n.a.	Ph1	small molecule		
HS-10535	GLP-1	Hansoh/MSD	n.a.	Ph1	small molecule		
conveglipron (HDM1002)	GLP-1	Huadong Medicine	Ph3	IND	small molecule	100-400mg, BID	
MDR-001	GLP-1	MindRank	Ph3	IND	small molecule	30-360mg, BID	
Ribupatide (HRS9531)	GLP-1/GIP	Hengrui/Kailera	Ph2	n.a.	peptide		
DA-302168S	GLP-1	DIAO Pharma	Ph2	n.a.	small molecule	15mg, QD	
HS-10501	GLP-1	Hansoh	Ph2	n.a.	small molecule	2.5-20mg, QD	
MWN109	GLP-1/GIP/GCGR	Minwei Biotech	Ph2	n.a.	peptide		
ZT006	GLP-1	QL Biopharm	Ph2	n.a.	peptide		
SAL0112	GLP-1	SalubrisBio	Ph1	n.a.	small molecule		
VRB-103	amylin	Sciwind	IND	n.a.	peptide	potential QW	
XW014	GLP-1	Sciwind	IND	n.a.	small molecule	up to 120/140mg, QD	
China asset							
Highlighted assets with data sharing at ADA2026							

As of May 2026

Source: PharmCube

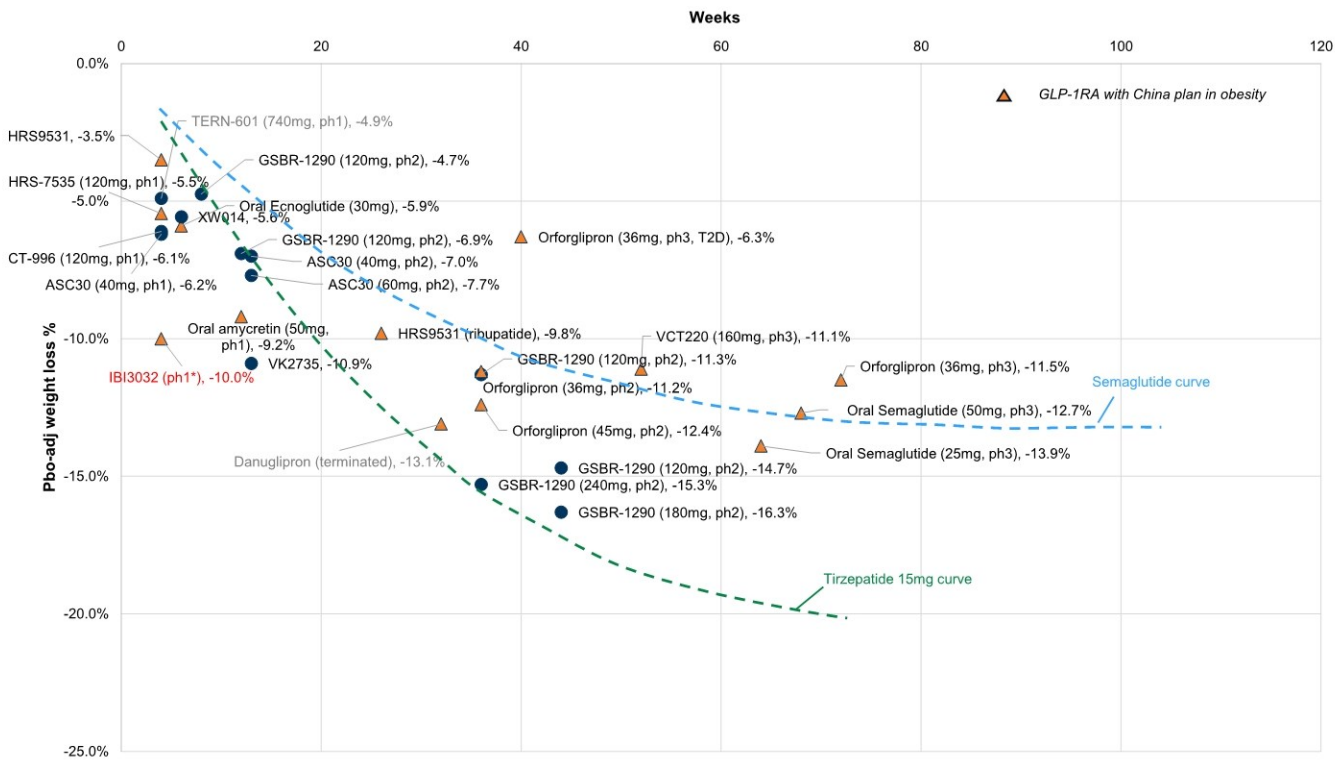
Wegovy pill / Foundayo ph3 data as the bar, eyes on tolerability with “lower and slower” strategy: We previously stated that (see [our China GLP-1 update on oral/injectable debates](#)) newer oral GLP-1 assets have not shown strong clinical evidence of clear competitive advantage over orforglipron, and with the approval of Wegovy pill / Foundayo, their ph3 data remain as competitive in both efficacy and tolerability: **1) weight loss**, which is not the major focus for oral GLP-1, given the less severe target groups and the KOL opinions that **10+% weight loss is acceptable** in primary care, and the 13.9% / 11.5% placebo-adjusted weight loss of Wegovy pill / Foundayo are already putting the bar to a relatively high position ([Exhibit 2](#)). To our knowledge, alenglipron (GPCR-1290) is the first oral GLP-1 that showed meaningfully higher weight loss with higher dose (-14.7%/-16.3%/-16% at 120mg/180mg/240mg in the ph2 ACCESS II trial), and Innovent briefly shared that IBI3032 achieved 10% weight loss in 4 weeks, which could be a significant differentiation with eyes on the data details at ADA2026; **2) tolerability**, which is the primary focus for oral GLP-1, though newer oral GLP-1 molecules are often designed with better PK profile that showed good control of GI toxicity in early-stage trials, we view the ph3 data of Wegovy pill / Foundayo as the ground that any new oral GLP-1 drug candidates are expected to beat, in particular, **vomiting as the most severe AE is expected to have <20% occurrence rate** ([Exhibit 3](#)). In addition, it is becoming clear that adopting a “lower and slower” dose titration strategy could effectively improve the tolerability (see our [note](#)), therefore putting more focus on the design of pivotal trials, while large variance of AE rates are typically accepted for early trials (e.g. SAD or MAD when the dose titration is still being explored).

Framing expectation for IBI3032 readout at ADA2026, pipeline value hinges on

global clinical execution: Per Innovent, the ph1 SAD / MAD data of IBI3032 will be shared at the ADA conference, and we expect to see more details on the drug profile, including: 1) sample size, though we do not expect the company to share data of all dosed patients (100+ per mgmt), 20-30% (i.e. N=20-30, vs. 24 patients in the MAD data for ASC30) would serve a relatively meaningful sample size to reduce the random variations, 2) dose dependency, while mgmt previously shared that 10% weight loss was reached with 4 weeks of treatment, we would want to know more about the corresponding dose levels (DL) to gain insights on the efficacy window, and 3) tolerability delta between SAD and MAD, while large variation for AE rates is expected given the early stage, we would expect trend of improved tolerability with MAD (vs. SAD), and to gain early colors on the potential dose titration scheme for IBI3032. Given the current landscape of oral GLP-1, we believe the value of IBI3032 to hinge on 1) the potential differentiation vs. later-stage peers, and 2) global clinical execution, noting that IBI3032 is roughly 2.5 years / 1.5 years behind aleniglipron / ASC30 (both targeting to initiate global ph3 trials in 2H26).

Emerging datasets from Hengrui to demonstrate portfolio breadth and potential synergies: At the upcoming ADA conference, Hengrui plans to present multiple datasets across its GLP-1 franchise, highlighting both oral and injectable solutions: 1) **HRS9531 (oral GLP-1/GIP peptide)** will disclose detailed phase 2 obesity data, following previously reported efficacy of -9.8% weight loss at week 26; we look for further details on titration scheme and dosing requirements to assess safety implications. Beyond obesity, HRS9531 will also present data in polycystic ovary syndrome, suggesting potential expansion into adjacent metabolic indications. 2) **HR17031 (GLP-1/insulin fixed-dose combination)** will report data versus insulin glargine in uncontrolled diabetes patients, indicating a push into combination therapy positioning. In addition, Hengrui's **oral small-molecule GLP-1 (HRS7535)** has met primary endpoints in phase 3 diabetes trial, with NDA filing planned for 2026. For next-generation multi-target agents, **HRS-4729 (GLP-1/GIP/GCG)** demonstrated -16% weight loss (-10.6% placebo-adjusted) at week 12 with tolerable safety. Based on non-head-to-head comparison, efficacy appears slightly lower than **UBT251 (United Labs/Novo Nordisk)**, which achieved -16.6%/-17.7% placebo-adjusted weight loss at week 12/24, and it is expected to release further phase 2 data at ADA. Overall, the breadth of datasets across oral, combination and multi-target modalities underscores Hengrui's strategy to build a full-spectrum GLP-1 portfolio, with differentiation likely to hinge on safety, convenience and commercial execution.

Exhibit 2: 15% weight loss is becoming the bar for newer oral GLP-1 after the approval of Wegovy pills and orforglipron...
Weight loss data of oral GLP-1 with different treatment duration (placebo-adjusted)

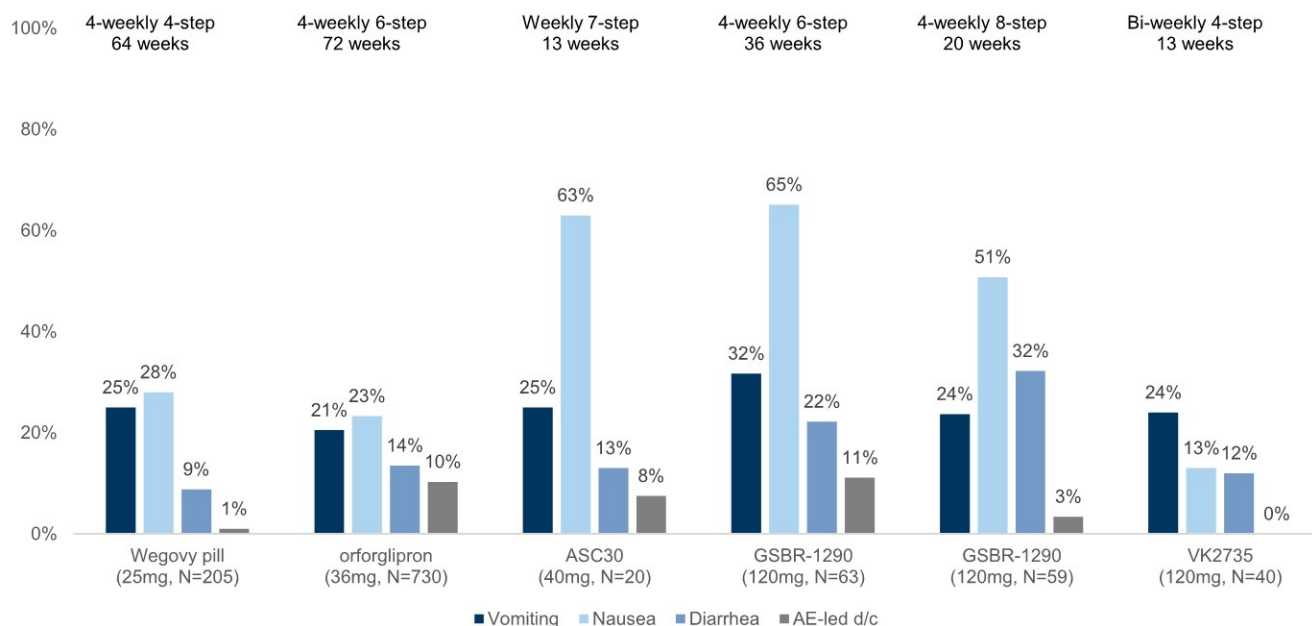


*treatment arm data (placebo unadjusted)

Source: Company data, Goldman Sachs Global Investment Research

Exhibit 3: ... though key focus remains on better tolerability with <20% vomiting rate

Safety profile of key oral GLP-1



Rate of adverse events are placebo-adjusted

Source: Company data

Investment Thesis - Hengrui

Hengrui is a leading pharma company in China that is in the process of transforming its business focus to innovative drugs from a “generics + innovation” model previously. Supported by an established strong sales team, Hengrui has launched 9 drugs with sales of Rmb1bn+ in the past few years, including 4 innovative drugs. With the majority of the impact from VBP price cuts largely reflected and reset to a lower base in 2022, we expect the future drivers for Hengrui to come from innovative drugs including PARP, CDK4/6, AR antagonist, PI3Ki and diabetes drugs. We are Buy rated as we believe the current valuation largely only reflects the company’s China prospects and overseas progress should present further upside. We expect growth in the current product cycle to slow down post the ramp-up of blockbuster PD-1, and that overseas progress will be the next key milestone for the company. Key risks: 1) a slower ramp-up of innovative drugs post the NRDL listing; 2) risk of failure in key late-stage R&D programs; 3) higher-than-expected R&D and admin expenses for global expansion; and 4) greater-than-expected price cuts for generics and innovative drugs.

Price Target Risks and Methodology - Hengrui Medicine

We have a Buy rating on Hengrui. Our 12-m TP of Rmb79.20 is based on: 1) a long-term exit P/E valuation for the generics business of Rmb40.4bn (based on a 5-year exit P/E of 10x); and 2) a risk-adjusted DCF for innovative drugs, deriving a valuation of Rmb420.7bn (based on WACC of 9.0%, terminal growth rate of 3%). Key risks: 1) a slower ramp-up of innovative drugs post the NRDL listing; 2) risk of failure in key late-stage R&D programs; 3) higher-than-expected R&D and admin expenses for global expansion; 4) greater-than-expected price cuts for generics and innovative drugs; 5)

below-expected license-out / go-global progress.

Investment Thesis - Innovent Biologics

Innovent Biologics is a leading China-based biopharma company with a strong presence in immuno-oncology and growing assets in autoimmune/metabolic/ophthalmology. The company has a deep pipeline of 30+ assets built via in-house R&D and collaboration with global partners, including commercialized products led by sintilimab (the leading PD-1 inhibitor covering major indications in the NRDL). We believe Innovent will continue its leading position in China biotech with: 1) deep pipeline of novel molecules targeting next-generation I/O targets with encouraging preliminary data (e.g., IBI363 (PD-1/IL-2 α -bias BsAb) with a differentiated drug profile in IO-treated / cold tumors, mazdutide (GLP-1/GCGR) with encouraging efficacy in obesity/diabetes); 2) strong commercialization capabilities as best demonstrated in the quick ramp-up of sintilimab; and 3) its deep collaboration with global partners, especially Eli Lilly. We consider Innovent as undervalued at current levels given the high value of market-implied WACC (12%) only reflecting the value of its de-risked indications and not fully pricing in its strong revenue and pipeline. We are Buy rated on the stock. Key risks include: 1) Intensifying competition in the PD-1/L1 market in China; 2) uncertain approval timeline for key candidates; 3) potential restrictions on off-label use arising from any safety issues; and 4) failure of R&D projects.

Price Target Risks and Methodology - Innovent Biologics

Our risk-adjusted, DCF-based, 12-month TP of HK\$107.04 is based on a: 1) 10% discount rate, and 2) 3% terminal growth rate. We have a Buy rating. Key risks include: 1) Intensifying competition in the PD-1/L1 market in China; 2) uncertain approval timeline for key candidates; 3) potential restrictions on off-label use arising from any safety issues; and 4) failure of R&D projects.

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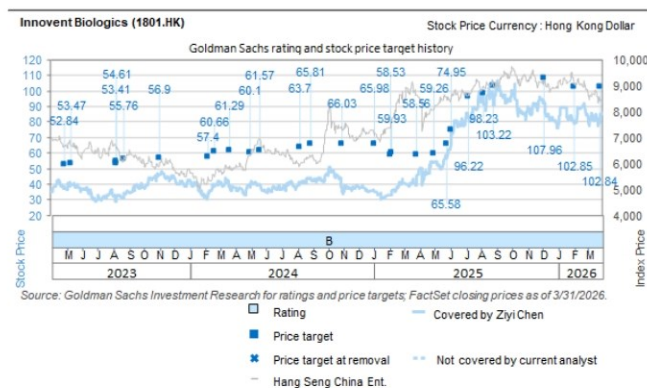
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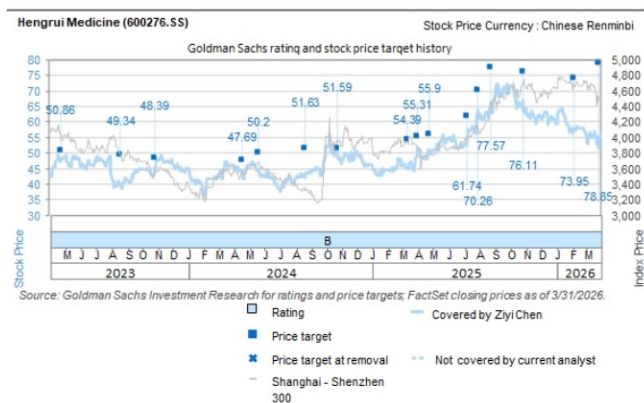
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Target price history table(s)

Innovent Biologics (1801.HK)

Date of report	Target price (HK\$)	Closing price (HK\$)
29-May-26	107.04	83.35
30-Apr-26	105.94	90.00
28-Apr-26	104.61	82.60
26-Mar-26	102.84	79.40
04-Feb-26	102.85	80.65
05-Dec-25	107.96	92.00
27-Aug-25	103.22	91.00
07-Aug-25	98.23	91.40
08-Jul-25	96.22	82.35
04-Jun-25	74.95	74.25
26-May-25	65.58	56.55
01-May-25	59.26	53.75
26-Mar-25	58.56	39.05
06-Feb-25	59.93	35.80
05-Feb-25	58.53	33.05
02-Jan-25	65.98	35.50
30-Oct-24	66.03	35.50
28-Aug-24	65.81	44.10
08-Aug-24	63.70	41.15
19-May-24	61.57	40.05
30-Apr-24	60.10	38.25
20-Mar-24	61.29	39.35
20-Feb-24	60.66	40.15
06-Feb-24	57.40	33.00
02-Nov-23	56.90	46.60
23-Aug-23	55.76	30.75
09-Aug-23	54.61	32.20
08-Aug-23	53.41	28.70

Hengrui Medicine (600276.SS)

Date of report	Target price (Rmb)	Closing price (Rmb)
22-Apr-26	79.20	55.44
26-Mar-26	78.85	51.63
05-Feb-26	73.95	58.16
28-Oct-25	76.11	63.62
24-Aug-25	77.57	64.98
29-Jul-25	70.26	63.80
08-Jul-25	61.74	53.40
24-Apr-25	55.90	51.13
31-Mar-25	55.31	49.20
11-Mar-25	54.39	45.34
24-Oct-24	51.59	48.67
22-Aug-24	51.63	43.93
19-May-24	50.20	44.75
17-Apr-24	47.69	44.39
26-Oct-23	48.39	44.45
20-Aug-23	49.34	39.95

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